This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

ISSUED, VALID FROM 30 SEPTEMBER 2026

Patient Group Direction

for the supply of Clobetasone Butyrate 0.05% or Betamethasone Valerate 0.1% for the treatment of Eczema and Dermatitis

Eczema and Dermatitis

From age 12 years onwards.

Patient Group Direction, version 008, issued 30 September 2026. Clobetasone butyrate 0.05% and betamethasone valerate 0.1%, 12 years and over.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD is for

The supply of a topical corticosteroid to a patient aged 12 years or over with a flare of eczema or dermatitis, by a registered pharmacist or registered pharmacy technician in a community pharmacy, without a prescription.

What it authorises

- Clobetasone butyrate 0.05% cream or ointment, a moderately potent corticosteroid, for mild eczema anywhere on the body, and for mild or moderate eczema on the face, flexures or genital skin for up to 7 days.
- Betamethasone valerate 0.1% cream or ointment, a potent corticosteroid, for moderate eczema on the trunk and limbs only.

What it does not cover

- The eyelids. Refer.
- Betamethasone valerate on the face, flexures or genital skin, where a potent steroid causes skin thinning quickly.
- Infected, weeping or crusted eczema, unless the infection is mild and localised and is treated at the same visit under the Skin and Soft Tissue Infection PGD. Otherwise refer.
- Children under 12.

Summary of NICE and NICE CKS guidance for eczema and dermatitis

NICE CG57, Atopic eczema in under 12s; NICE Clinical Knowledge Summaries, Eczema atopic; MHRA Drug Safety Update on the fire risk with emollients. Summarised 9 September 2026.

Overview

Eczema, or dermatitis, is a chronic relapsing inflammatory skin condition characterised by itch, dryness and inflammation.

Common types include atopic eczema, contact dermatitis (irritant or allergic), seborrhoeic dermatitis, and discoid and varicose eczema.

Assess severity, distribution, triggers and the effect on sleep and daily life. Look for signs of infection: weeping, crusting, or sudden worsening.

Emollients are the foundation

Emollients are first line for everyone, used liberally and frequently, and continued even when the skin is clear.

Ointments for dry skin, creams and lotions where a greasy product is not tolerated.

- **FIRE RISK: emollients, including paraffin-free products, soak into clothing and bedding and make them burn faster. This is an MHRA safety issue and must be counselled on every time.**

Topical corticosteroids

Used for flares, at the lowest potency that will control the flare and for the shortest time.

- Mild: hydrocortisone 1%. Moderate: clobetasone butyrate 0.05%. Potent: betamethasone valerate 0.1%.
- Potency is chosen by SEVERITY and by SITE. Thin skin, meaning the face, eyelids, flexures and genital skin, needs a lower potency and a shorter course.
- Step up and down according to response. Measure the amount in fingertip units.

What must be excluded before a steroid is supplied

Untreated fungal infection. A topical steroid on tinea produces tinea incognito: the rash spreads, loses its defined edge and becomes much harder to diagnose.

Eczema herpeticum: rapidly worsening, painful, punched-out or clustered vesicular lesions, or a systemically unwell patient. This is an emergency.

Rosacea, perioral dermatitis and acne, all of which a topical steroid makes worse.

Infected eczema

Signs are weeping, crusting, sudden worsening or fever.

Mild localised infection may be treated alongside the corticosteroid; this PGD provides for that jointly with the Skin and Soft Tissue Infection PGD.

Widespread or systemic infection needs an oral antibiotic and assessment, not a topical steroid alone.

The Patient Group Direction

How to use this PGD

Which arm, decided by severity AND site

BETAMETHASONE VALERATE 0.1% (Arm 2) IS NOT AUTHORISED on the face, eyelids, flexures or genital skin. A potent steroid on thin skin causes atrophy quickly.

CLOBETASONE BUTYRATE 0.05% (Arm 1) MAY BE USED ON THOSE SITES, for MILD OR MODERATE disease, FOR UP TO 7 DAYS ONLY at those sites.

THE EYELIDS ARE EXCLUDED FROM BOTH ARMS. Refer.

So: moderate eczema on the face, flexures or genital skin is treated with clobetasone under Arm 1, capped at 7 days.

MILD means limited erythema and scaling, not markedly affecting sleep or daily activity. MODERATE means marked erythema or lichenification, or disease disturbing sleep or daily activity. SEVERE means widespread erythema with oozing, bleeding or cracking of the skin, or disease seriously disturbing sleep or daily activity (NICE CG57 grading): severe disease is outside this PGD, refer. Excoriation from scratching may be present in either mild or moderate disease and does not by itself make disease moderate.

Concurrent supply for infected eczema

Where eczema is complicated by MILD, LOCALISED secondary bacterial infection, a patient may receive a topical corticosteroid under this PGD and an oral antibiotic under the Skin and Soft Tissue Infection PGD at the same consultation, both recorded in one consultation record. Where the infection is not mild and localised, or any red flag from the infection PGD is present, refer and supply neither.

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Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

Patient Group Direction

for the supply, for the treatment of Eczema and Dermatitis, of:

Arm 1. Clobetasone butyrate 0.05%, moderate potency

Patient Group Direction for the supply of clobetasone butyrate 0.05% cream or ointment for MILD eczema or dermatitis at any permitted site, and for MODERATE eczema or dermatitis on the face, flexures or genital skin, in patients aged 12 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be able to grade eczema severity as mild or moderate against the definitions in this document, since severity and site together decide which arm applies.
- Must be able to distinguish eczema and dermatitis from the conditions excluded below, in particular ROSACEA, PERIORAL DERMATITIS, ACNE, TINEA (fungal infection) and ECZEMA HERPETICUM.
- Must be able to apply the fingertip unit conversion in Appendix 1 to size a supply to the treated area.
- Must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Short-term treatment of eczema or dermatitis where a topical corticosteroid is appropriate: mild disease at any permitted site, and moderate disease on the face, flexures or genital skin where betamethasone valerate 0.1% is not authorised. First-line potency under this PGD.
Which arm applies	**BETAMETHASONE VALERATE 0.1% (Arm 2) IS NOT AUTHORISED on the face, eyelids, flexures or genital skin. A potent steroid on thin skin causes atrophy quickly.** **CLOBETASONE BUTYRATE 0.05% (Arm 1) MAY BE USED ON THOSE SITES, for MILD OR MODERATE disease, FOR UP TO 7 DAYS ONLY at**

	those sites. **THE EYELIDS ARE EXCLUDED FROM BOTH ARMS. Refer.** **So: moderate eczema on the face, flexures or genital skin is treated with clobetasone under Arm 1, capped at 7 days.**
Inclusion criteria	● Aged 12 years and over. ● **EITHER mild eczema or dermatitis at any permitted site: limited erythema and scaling, not markedly affecting sleep or daily activity. On the face, flexures or genital skin the 7 day cap applies whatever the severity.** ● **OR moderate eczema or dermatitis ON THE FACE, FLEXURES OR GENITAL SKIN, where the potent arm is not authorised. Treatment at those sites is capped at 7 days.** ● A diagnosis of eczema or dermatitis that the patient or their carer recognises, or that is clear on examination. ● Affected area no more than 10% of body surface, about ten adult palms. ● **Valid informed consent given. Where the patient is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent.** ● No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following is present.** ● Under 12 years of age. ● **Application to the EYELIDS. Refer.** ● **UNTREATED FUNGAL INFECTION, or any rash that might be tinea. A topical steroid on tinea produces tinea incognito: the rash spreads, loses its edge and becomes much harder to diagnose.** ● **SUSPECTED ECZEMA HERPETICUM: rapidly worsening, painful, punched-out or clustered vesicular lesions, or the patient is systemically unwell. This is an emergency, not a routine referral.** ● Signs of secondary bacterial infection, unless being treated concurrently under the Skin and Soft Tissue Infection PGD. See the note on concurrent supply. ● Ulcerated skin, or an open wound. ● Rosacea, perioral dermatitis or acne. A topical steroid makes all three worse. ● More than 10% of body surface affected, about ten adult palms. Refer. ● Three or more courses already supplied in the last 12 months without GP review. ● Known hypersensitivity to the product or any excipient. ● Pregnancy or breastfeeding, where treatment would be to the breast or nipple area. Otherwise short-term use of these potencies is acceptable; record the site. ● **MODERATE disease on the trunk or limbs. Use Arm 2.**
Broken, weeping and excoriated skin	**ULCERATED SKIN AND OPEN WOUNDS ARE EXCLUDED. Refer.** **EXCORIATION FROM SCRATCHING IS NOT AN EXCLUSION.** **WEEPING OR CRUSTED SKIN suggests secondary bacterial infection. Handle it under the concurrent supply route below, or refer.**
Cautions	Prolonged use, particularly on the face, flexures and genital skin, causes skin thinning, striae and telangiectasia. That is why those sites are capped at 7 days. Do not use under occlusion unless specifically advised, and do not apply to

	skin under a nappy or a tight dressing. A flare that has not settled after 7 days needs review rather than a longer course. Where the patient is already using another topical corticosteroid, do not add a second. Refer.
Actions if excluded or declines	• Where the exclusion is eyelid involvement, or a rash that may be fungal, say plainly why a steroid is not the right treatment and arrange review. • **Where eczema herpeticum is suspected, arrange EMERGENCY assessment, not a routine referral.** • Where the area is above 10% of body surface, or the patient has had three courses in 12 months, refer to the GP for review rather than supplying again. • Document the reason for exclusion, the advice given and the decision reached.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	• **That valid informed consent was given. Where the patient is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis recorded.** • Patient name, address, date of birth, and the GP with whom they are registered. • **The SEVERITY graded as mild or moderate, and the SITE treated. Together these decide the arm, the duration cap and the quantity.** • **The treated area in adult palms or fingertip units, and the quantity supplied against it.** • **Where a face, flexure or genital site was treated, that the 7 day cap was explained and recorded.** • How many courses the patient has had in the last 12 months. • Where a concurrent antibiotic was supplied for secondary infection, that both supplies are in one consultation record. • **That the fire risk from emollients was explained.** • Name and registration number of the healthcare professional supplying. • Name of the medicine, date of supply, dose, form, route and quantity supplied, with batch number and expiry date. • Advice given, including advice given if excluded or declining treatment. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.

The medicine

Name, form and strength	Clobetasone butyrate 0.05% cream or ointment. Ointment for dry, lichenified skin; cream for moist areas such as the flexures and for the face where an ointment is not tolerated. Weeping skin suggests secondary infection and is handled under the exclusion and concurrent supply provisions, not by choosing the cream.
Legal category	POM.

Route and method	Topical. Apply a THIN layer to affected skin only, once or twice daily. Measure the amount in fingertip units using Appendix 1.
Dose and frequency	Apply a thin layer once or twice daily to affected skin only.
Quantity to be supplied	**Sized to the treated area for the initial 7 days, with margin.** Working, using the conversion in Appendix 1: one fingertip unit is about 0.5g and covers about two adult palms. ● UP TO 2 ADULT PALMS, about 2% of body surface. 1 fingertip unit, 0.5g per application, 1g a day. SUPPLY ONE 30g TUBE, the smallest marketed pack. ● 2 TO 5 ADULT PALMS. Up to 2.5 fingertip units, 1.25g per application, 2.5g a day. SUPPLY ONE 30g TUBE. ● 5 TO 10 ADULT PALMS, up to the 10% maximum. Up to 5 fingertip units, 2.5g per application, 5g a day, so 35g over 7 days. SUPPLY TWO 30g TUBES (60g). ● Both products are marketed as 30g and 100g tubes. Supply 30g tubes only, one or two by treated area as above; do not supply a 100g tube, and do not record a pack size that does not exist. Record the number of tubes supplied. **ABOVE 10% OF BODY SURFACE, about ten adult palms: refer rather than supply. That has not changed.**
Maximum treatment period	**ON THE FACE, FLEXURES OR GENITAL SKIN: 7 DAYS MAXIMUM, whichever arm and whichever severity. Not 4 weeks.** ON THE TRUNK AND LIMBS: up to 7 days initially, then review. Maximum 4 weeks of continuous daily treatment in total. Longer than that requires GP review. Maximum three courses in any 12 months before GP review. One supply per consultation. A second supply may be made after review, within the 4 week ceiling, using the same area-based quantities as the initial supply and covering no more than the days remaining within that ceiling.
Adverse effects	Common with prolonged use: skin thinning, striae, telangiectasia, local burning or itching on application. Uncommon: folliculitis, hypopigmentation, worsening of an unrecognised infection. Rare: systemic absorption with extensive or occluded use, contact dermatitis to an excipient.
Storage	Store below 25C. Do not use after the expiry date. Discard the tube 3 months after opening.

Information for the patient

Written information	Supply the patient information leaflet, and show the patient how to measure a fingertip unit on their own finger.
Counselling	● **Apply the steroid FIRST, in a thin layer, to the affected skin only. WAIT AT LEAST 30 MINUTES, then apply your emollient. Do not put emollient straight over a freshly applied steroid: it dilutes it and spreads it onto skin that does not need it.** ● Use the fingertip unit measure we have shown you. One fingertip unit covers about two adult palms. ● On the face, in skin folds or on genital skin, use this for no more than 7 days. ● Keep using your emollient every day, including after the steroid course finishes. The emollient is the treatment that keeps eczema away; the

	steroid settles a flare. **FIRE RISK FROM EMOLLIENTS. All emollients, including paraffin-free ones, soak into clothing, bedding and dressings and make them catch fire more easily and burn faster. Tell every patient not to smoke, use a naked flame, or go near anything burning, and to wash clothing and bedding often, knowing that washing may not remove the residue completely.** ● Come back or see your GP if it is no better after 7 days, if it spreads, or if it starts weeping, crusting or becoming painful. ● **Seek urgent help if the rash becomes rapidly painful with clustered blisters or punched-out sores, or you feel unwell with it.**
Follow-up	Review if not improved after 7 days, or sooner if the rash spreads, weeps, crusts or becomes painful. Beyond 4 weeks of continuous treatment on the trunk or limbs, or 7 days on the face, flexures or genital skin, the patient needs GP review rather than a further supply here.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- NICE CG57, Atopic eczema in under 12s; NICE Clinical Knowledge Summaries, Eczema atopic; MHRA Drug Safety Update on the fire risk with emollients. Summarised 9 September 2026.
- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Authorisation of this version

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v008
Supersedes	Version 007, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
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Valid from date	30 September 2026
Expiry date	31 July 2027

GET REAL HEALTH

Change history to version 004

Later versions are recorded in the Version and change record at the end of this document.

Version	v004
Date	9 September 2026
Changes	● The summary of the governing guidance is restored as part 2 of the document, which is the house format for every Get Real Health PGD: what the PGD is for and which medicines it authorises, then a summary of the guidance that governs the condition, then the PGD itself. Twenty documents lost that section in the September 2026 rewrites, because the generator did not emit it and no check looked for it. It is the section that lets a pharmacist hold the PGD against the guidance it claims to follow, which is exactly how the inverted dental indication should have been caught. It is now a required field: the generator refuses to build a document without it. ● MODERATE ECZEMA ON THE FACE, FLEXURES OR GENITAL SKIN NOW HAS A SUPPLY ROUTE. Version 002 told staff, on the scope page and again in Arm 2, to supply clobetasone under Arm 1 for that patient, while Arm 1 admitted only MILD disease. The patient therefore had no route. Arm 1 now covers mild disease at any permitted site and moderate disease at those sites, capped at 7 days. Raised by an adopting pharmacy. ● THE FACIAL DURATION CONTRADICTION IS RESOLVED. The 7 day cap for clobetasone on the face, flexures and genital skin appeared only inside an exclusion bullet, while the maximum treatment period box for the same arm permitted 4 weeks of continuous daily treatment. The cap is now in the duration box, the counselling and the records. ● QUANTITIES ARE SIZED TO THE TREATED AREA, with the arithmetic shown in Appendix 1. Version 002 offered 15g and 30g against a maximum treatable area of 10% of body surface; at 2.5g per application twice daily the 30g maximum lasts six days, against a document permitting four weeks. Supplies are now 15g, 30g and 60g by area. ● THE EXCORIATION CONTRADICTION IS RESOLVED. Arm 2 listed excoriation as a marker of moderate disease qualifying a patient, and excluded broken skin three bullets below. Excoriation is broken skin. Excoriation is now expressly not an exclusion, ulcerated skin and open wounds are, and weeping or crusted skin routes to the concurrent supply provision or to referral. Version 001 excluded weeping skin and version 002 dropped the word without recording it. ● FUNGAL AND VIRAL INFECTION ARE BACK IN THE EXCLUSIONS, and tinea is back in the differential that staff must be trained on. Version 001 excluded untreated fungal, bacterial and viral skin infection; version 002 covered bacterial infection and eczema herpeticum only. A topical steroid on tinea produces tinea incognito, and registered technicians are authorised to supply a potent corticosteroid under this PGD. ● THE FIRE RISK WARNING IS RESTORED, with its own appendix and a requirement to record that it was explained. Emollients, including paraffin-free ones, soak into fabric and make it burn faster. Version 001 carried the warning and version 002 lost it. ● AN INTERVAL BETWEEN STEROID AND EMOLLIENT IS STATED: at least 30 minutes. Version 002 said only "wait for it to absorb". ● The reversal of the application order is now recorded. Version 001 applied the steroid after emollients; version 002 reversed it to steroid first and said nothing about having done so. Version 003 keeps steroid first and explains why.

	• Severity definitions are stated once at the front and referenced by both arms, and it is made explicit that excoriation does not by itself make disease moderate. • Consent in children and young people added: parental responsibility, or Gillick competence with the basis recorded. • The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored, together with the standard governance requirements: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent. All were in version 001 and all were lost in the version 002 rewrite. • The Key references section is restored, lost in the version 002 rewrite. • Records must now carry the severity, the site, the treated area in palms or fingertip units, the quantity against it, the 7 day cap where a face, flexure or genital site was treated, and that the fire risk was explained. • Topical calcineurin inhibitors (tacrolimus, pimecrolimus) and the topical and oral antibiotic content in the version 001 summary pages are recorded here as deliberate removals. Calcineurin inhibitors require specialist initiation and are outside a PGD; antibiotic treatment of infected eczema is handled by the Skin and Soft Tissue Infection PGD through the concurrent supply route. Both were dropped in version 002 and neither appeared in any change log. • This change history is itemised. Version 002 recorded only "Full clinical review and reissue", which is why an adopting pharmacy had to compare the two versions line by line to find the contradictions above.

Previous versions

003, 9 September 2026	See the change history of that version. Superseded the same day by v004, which restores the guidance summary section.
002, 7 September 2026	Full clinical review and reissue. Restructured into a clobetasone arm and a betamethasone valerate 0.1% arm, concurrent supply for infected eczema introduced, pimecrolimus, hydrocortisone and fusidic acid removed. Introduced the contradictions corrected in 003.
001, 1 November 2025	Development and issue of new PGD.

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

Patient Group Direction

for the supply, for the treatment of Eczema and Dermatitis, of:

Arm 2. Betamethasone valerate 0.1%, potent

Patient Group Direction for the supply of betamethasone valerate 0.1% cream or ointment for MODERATE eczema or dermatitis on the TRUNK AND LIMBS ONLY, in patients aged 12 years and over.

Not for the face, eyelids, flexures or genital skin. For moderate disease at those sites, use clobetasone under Arm 1, capped at 7 days.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be able to grade eczema severity as mild or moderate against the definitions in this document, since severity and site together decide which arm applies.
- Must be able to distinguish eczema and dermatitis from the conditions excluded below, in particular ROSACEA, PERIORAL DERMATITIS, ACNE, TINEA (fungal infection) and ECZEMA HERPETICUM.
- Must be able to apply the fingertip unit conversion in Appendix 1 to size a supply to the treated area.
- Must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Short-term treatment of MODERATE eczema or dermatitis on the trunk and limbs, where a moderate-potency steroid is insufficient or the presentation is clearly moderate at first assessment.
Which arm applies	**BETAMETHASONE VALERATE 0.1% (Arm 2) IS NOT AUTHORISED on the face, eyelids, flexures or genital skin. A potent steroid on thin skin causes atrophy quickly.** **CLOBETASONE BUTYRATE 0.05% (Arm 1) MAY BE USED ON THOSE**

	SITES, for MILD OR MODERATE disease, FOR UP TO 7 DAYS ONLY at those sites. **THE EYELIDS ARE EXCLUDED FROM BOTH ARMS. Refer.** **So: moderate eczema on the face, flexures or genital skin is treated with clobetasone under Arm 1, capped at 7 days.**
Inclusion criteria	● Aged 12 years and over. ● **MODERATE eczema or dermatitis: marked erythema or lichenification, or disease disturbing sleep or daily activity.** ● **ON THE TRUNK OR LIMBS ONLY.** ● Affected area no more than 10% of body surface, about ten adult palms. ● Valid informed consent given, and where under 16, on the basis set out in Arm 1. ● No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following is present.** ● Under 12 years of age. ● **Application to the EYELIDS. Refer.** ● **UNTREATED FUNGAL INFECTION, or any rash that might be tinea. A topical steroid on tinea produces tinea incognito: the rash spreads, loses its edge and becomes much harder to diagnose.** ● **SUSPECTED ECZEMA HERPETICUM: rapidly worsening, painful, punched-out or clustered vesicular lesions, or the patient is systemically unwell. This is an emergency, not a routine referral.** ● Signs of secondary bacterial infection, unless being treated concurrently under the Skin and Soft Tissue Infection PGD. See the note on concurrent supply. ● Ulcerated skin, or an open wound. ● Rosacea, perioral dermatitis or acne. A topical steroid makes all three worse. ● More than 10% of body surface affected, about ten adult palms. Refer. ● Three or more courses already supplied in the last 12 months without GP review. ● Known hypersensitivity to the product or any excipient. ● Pregnancy or breastfeeding, where treatment would be to the breast or nipple area. Otherwise short-term use of these potencies is acceptable; record the site. ● **ANY application to the face, eyelids, flexures or genital skin. Use Arm 1 for those sites.** ● **Mild disease. Use Arm 1: do not start at a potent steroid where a moderate one is appropriate.**
Broken, weeping and excoriated skin	**ULCERATED SKIN AND OPEN WOUNDS ARE EXCLUDED. Refer.** **EXCORIATION FROM SCRATCHING IS NOT AN EXCLUSION. It is expected in eczema, and version 002 both listed it as a marker of MODERATE disease qualifying a patient for Arm 2 and excluded "broken or ulcerated skin" three bullets below, so the same finding included and excluded the same patient.** **WEEPING OR CRUSTED SKIN suggests secondary bacterial infection. Handle it under the concurrent supply route below, or refer.**
Cautions	Potent steroids cause atrophy faster than moderate ones. Keep the course as short as the flare requires and review at 7 days.

	Do not use under occlusion unless specifically advised. Where the patient is already using another topical corticosteroid, do not add a second. Refer. Step down to a moderate potency once the flare settles rather than stopping abruptly, and say so to the patient.
Actions if excluded or declines	As Arm 1. Where the exclusion is site, supply clobetasone under Arm 1 instead where that is appropriate, and record which arm was used and why.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	● **That valid informed consent was given. Where the patient is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis recorded.** ● Patient name, address, date of birth, and the GP with whom they are registered. ● **The SEVERITY graded as mild or moderate, and the SITE treated. Together these decide the arm, the duration cap and the quantity.** ● **The treated area in adult palms or fingertip units, and the quantity supplied against it.** ● **Where a face, flexure or genital site was treated, that the 7 day cap was explained and recorded.** ● How many courses the patient has had in the last 12 months. ● Where a concurrent antibiotic was supplied for secondary infection, that both supplies are in one consultation record. ● **That the fire risk from emollients was explained.** ● Name and registration number of the healthcare professional supplying. ● Name of the medicine, date of supply, dose, form, route and quantity supplied, with batch number and expiry date. ● Advice given, including advice given if excluded or declining treatment. ● Details of any adverse drug reactions and the actions taken. ● That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.

The medicine

Name, form and strength	Betamethasone valerate 0.1% cream or ointment. Ointment for dry, lichenified skin; cream for moist areas.
Legal category	POM.
Route and method	Topical. Apply a THIN layer to affected skin only, once or twice daily. Measure in fingertip units using Appendix 1.
Dose and frequency	Apply a thin layer once or twice daily to affected skin only.
Quantity to be supplied	**Sized to the treated area for the initial 7 days, with margin.** Working, using the conversion in Appendix 1: one fingertip unit is about 0.5g and covers about two adult palms. ● UP TO 2 ADULT PALMS, about 2% of body surface. 1 fingertip unit,

	0.5g per application, 1g a day. SUPPLY ONE 30g TUBE, the smallest marketed pack. ● 2 TO 5 ADULT PALMS. Up to 2.5 fingertip units, 1.25g per application, 2.5g a day. SUPPLY ONE 30g TUBE. ● 5 TO 10 ADULT PALMS, up to the 10% maximum. Up to 5 fingertip units, 2.5g per application, 5g a day, so 35g over 7 days. SUPPLY TWO 30g TUBES (60g). ● Both products are marketed as 30g and 100g tubes. Supply 30g tubes only, one or two by treated area as above; do not supply a 100g tube, and do not record a pack size that does not exist. Record the number of tubes supplied. **ABOVE 10% OF BODY SURFACE, about ten adult palms: refer rather than supply. That has not changed.**
Maximum treatment period	Up to 7 days initially, then review. Maximum 4 weeks of continuous daily treatment on the trunk and limbs. Longer requires GP review. **This arm is not authorised on the face, flexures or genital skin at all, so the 7 day site cap does not arise here. If eczema appears at those sites, use Arm 1.** Maximum three courses in any 12 months before GP review.
Adverse effects	As Arm 1, with a higher risk of skin thinning, striae and telangiectasia because the potency is greater. Rare: systemic absorption with extensive or occluded use, and adrenal suppression with prolonged large-area use.
Storage	Store below 25C. Do not use after the expiry date. Discard the tube 3 months after opening.

Information for the patient

Written information	Supply the patient information leaflet, and show the patient how to measure a fingertip unit.
Counselling	● **Apply the steroid FIRST, in a thin layer, to the affected skin only. WAIT AT LEAST 30 MINUTES, then apply your emollient. Do not put emollient straight over a freshly applied steroid: it dilutes it and spreads it onto skin that does not need it.** ● Use the fingertip unit measure we have shown you. One fingertip unit covers about two adult palms. ● Do NOT use this on the face, eyelids, skin folds or genital skin. If eczema appears there, come back and we will look at it. ● Keep using your emollient every day, including after the steroid course finishes. The emollient is the treatment that keeps eczema away; the steroid settles a flare. **FIRE RISK FROM EMOLLIENTS. All emollients, including paraffin-free ones, soak into clothing, bedding and dressings and make them catch fire more easily and burn faster. Tell every patient not to smoke, use a naked flame, or go near anything burning, and to wash clothing and bedding often, knowing that washing may not remove the residue completely.** ● Come back or see your GP if it is no better after 7 days, if it spreads, or if it starts weeping, crusting or becoming painful. ● **Seek urgent help if the rash becomes rapidly painful with clustered blisters or punched-out sores, or you feel unwell with it.**
Follow-up	As Arm 1.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- NICE CG57, Atopic eczema in under 12s; NICE Clinical Knowledge Summaries, Eczema atopic; MHRA Drug Safety Update on the fire risk with emollients. Summarised 9 September 2026.
- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Authorisation of this version

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v008
Supersedes	Version 007, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
Valid from date	30 September 2026
Expiry date	31 July 2027

GET REAL HEALTH

Change history to version 004

Later versions are recorded in the Version and change record at the end of this document.

Version	v004
Date	9 September 2026
Changes	● The summary of the governing guidance is restored as part 2 of the document, which is the house format for every Get Real Health PGD: what the PGD is for and which medicines it authorises, then a summary of the guidance that governs the condition, then the PGD itself. Twenty documents lost that section in the September 2026 rewrites, because the generator did not emit it and no check looked for it. It is the section that lets a pharmacist hold the PGD against the guidance it claims to follow, which is exactly how the inverted dental indication should have been caught. It is now a required field: the generator refuses to build a document without it. ● MODERATE ECZEMA ON THE FACE, FLEXURES OR GENITAL SKIN NOW HAS A SUPPLY ROUTE. Version 002 told staff, on the scope page and again in Arm 2, to supply clobetasone under Arm 1 for that patient, while Arm 1 admitted only MILD disease. The patient therefore had no route. Arm 1 now covers mild disease at any permitted site and moderate disease at those sites, capped at 7 days. Raised by an adopting pharmacy. ● THE FACIAL DURATION CONTRADICTION IS RESOLVED. The 7 day cap for clobetasone on the face, flexures and genital skin appeared only inside an exclusion bullet, while the maximum treatment period box for the same arm permitted 4 weeks of continuous daily treatment. The cap is now in the duration box, the counselling and the records. ● QUANTITIES ARE SIZED TO THE TREATED AREA, with the arithmetic shown in Appendix 1. Version 002 offered 15g and 30g against a maximum treatable area of 10% of body surface; at 2.5g per application twice daily the 30g maximum lasts six days, against a document permitting four weeks. Supplies are now 15g, 30g and 60g by area. ● THE EXCORIATION CONTRADICTION IS RESOLVED. Arm 2 listed excoriation as a marker of moderate disease qualifying a patient, and excluded broken skin three bullets below. Excoriation is broken skin. Excoriation is now expressly not an exclusion, ulcerated skin and open wounds are, and weeping or crusted skin routes to the concurrent supply provision or to referral. Version 001 excluded weeping skin and version 002 dropped the word without recording it. ● FUNGAL AND VIRAL INFECTION ARE BACK IN THE EXCLUSIONS, and tinea is back in the differential that staff must be trained on. Version 001 excluded untreated fungal, bacterial and viral skin infection; version 002 covered bacterial infection and eczema herpeticum only. A topical steroid on tinea produces tinea incognito, and registered technicians are authorised to supply a potent corticosteroid under this PGD. ● THE FIRE RISK WARNING IS RESTORED, with its own appendix and a requirement to record that it was explained. Emollients, including paraffin-free ones, soak into fabric and make it burn faster. Version 001 carried the warning and version 002 lost it. ● AN INTERVAL BETWEEN STEROID AND EMOLLIENT IS STATED: at least 30 minutes. Version 002 said only "wait for it to absorb". ● The reversal of the application order is now recorded. Version 001 applied the steroid after emollients; version 002 reversed it to steroid first and said nothing about having done so. Version 003 keeps steroid first and explains why.

	• Severity definitions are stated once at the front and referenced by both arms, and it is made explicit that excoriation does not by itself make disease moderate. • Consent in children and young people added: parental responsibility, or Gillick competence with the basis recorded. • The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored, together with the standard governance requirements: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent. All were in version 001 and all were lost in the version 002 rewrite. • The Key references section is restored, lost in the version 002 rewrite. • Records must now carry the severity, the site, the treated area in palms or fingertip units, the quantity against it, the 7 day cap where a face, flexure or genital site was treated, and that the fire risk was explained. • Topical calcineurin inhibitors (tacrolimus, pimecrolimus) and the topical and oral antibiotic content in the version 001 summary pages are recorded here as deliberate removals. Calcineurin inhibitors require specialist initiation and are outside a PGD; antibiotic treatment of infected eczema is handled by the Skin and Soft Tissue Infection PGD through the concurrent supply route. Both were dropped in version 002 and neither appeared in any change log. • This change history is itemised. Version 002 recorded only "Full clinical review and reissue", which is why an adopting pharmacy had to compare the two versions line by line to find the contradictions above.

Previous versions

003, 9 September 2026	See the change history of that version. Superseded the same day by v004, which restores the guidance summary section.
002, 7 September 2026	Full clinical review and reissue. Restructured into a clobetasone arm and a betamethasone valerate 0.1% arm, concurrent supply for infected eczema introduced, pimecrolimus, hydrocortisone and fusidic acid removed. Introduced the contradictions corrected in 003.
001, 1 November 2025	Development and issue of new PGD.

Appendix 1: Fingertip units, treated area and quantity

- ONE FINGERTIP UNIT is the amount squeezed from a standard nozzle onto the pad of an adult index finger, from the tip to the first crease. It is about 0.5g and covers about TWO ADULT PALMS.
- Two adult palms is roughly 2% of body surface.
- **TEN ADULT PALMS is about 10% of body surface, which is the MAXIMUM area treatable under this PGD. Above that, refer.**

Quantity, worked from those conversions at twice-daily application over 7 days:

- Up to 2 palms: 1 fingertip unit, 0.5g per application, 1g a day, 7g a week. Supply one 30g tube.
- 2 to 5 palms: up to 2.5 fingertip units, 1.25g per application, 2.5g a day, 17.5g a week. Supply one 30g tube.
- 5 to 10 palms: up to 5 fingertip units, 2.5g per application, 5g a day, 35g a week. Supply two 30g tubes (60g).

Appendix 2: Steroid and emollient, order and interval

- Apply the STEROID FIRST, thinly, to affected skin only.
- **WAIT AT LEAST 30 MINUTES.**
- Then apply the emollient, generously, to the whole area including unaffected skin.
- Applying emollient straight over a freshly applied steroid dilutes it and carries it onto skin that does not need it.

What matters most in practice is that the two are separated and that the patient does the same thing each time.

Appendix 3: Fire risk from emollients

This warning applies to every patient using an emollient, which is every patient under this PGD.

- Emollients soak into clothing, bedding, dressings and upholstery.
- Fabric with dried-on emollient catches fire more easily and burns faster. This applies to paraffin-based AND paraffin-free products.
- Tell the patient not to smoke, use a naked flame, or go near anything burning.
- Wash clothing and bedding frequently, and understand that washing may not remove the residue completely.
- Record that this was explained.

Appendix 4: Key references

- NICE Clinical Knowledge Summaries: Eczema atopic, current revision.
- NICE CG57 Atopic eczema in under 12s, for the fingertip unit and severity definitions.
- MHRA Drug Safety Update on the fire risk associated with emollients, including paraffin-free products.
- Summary of Product Characteristics for clobetasone butyrate 0.05% and betamethasone valerate 0.1%, current versions.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

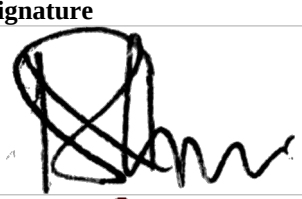
Version and change record
Version: v008
Issued: 30 September 2026
Supersedes: Version 007, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v007
Issued: 14 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Supply quantities in both arms are now stated as marketed packs: one 30g tube for a treated area of up to 5 adult palms, two 30g tubes for 5 to 10 palms. The earlier version directed a 15g supply for the smallest area and a 60g supply for the largest; betamethasone valerate 0.1% and clobetasone butyrate 0.05% (POM) are marketed as 30g and 100g tubes only, so neither could be dispensed as written. The 7 day and 4 week treatment ceilings are unchanged. Raised by an adopting pharmacy, 14 September 2026.
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Severe eczema is now defined (NICE CG57 grading) as outside this PGD, so the referral has a definition to test against.
Arm 1 mild-disease inclusion bullet now states the 7 day cap on the face, flexures or genital skin whatever the severity, matching the maximum treatment period row.
Clobetasone form row no longer names weeping areas as a reason to choose the cream, since weeping skin is treated as a sign of infection elsewhere in the document.
Second supply after review now uses the same area-based quantities as the initial supply and covers no more than the days remaining within the 4 week ceiling.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 9 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual

customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.